

ETHIOPIAN HEALTH PROFESSIONALS LICENSING EXAMINATION(EHPLE) PRACTICE TEST BOOKLET 2025

MEDICINE



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MINISTRY OF HEALTH-ETHIOPIA

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HEALTHIER CITIZENS FOR PROSPEROUS NATION!



Message from the State Minister, Ministry of Health – Ethiopia



Mrs. Frehiwot Abebe

Improving the quality of healthcare remains a critical global priority and is essential for achieving sustainable development and universal health coverage. One of the most effective ways to ensure high standards in healthcare delivery is through the consistent assessment of professional competencies. In line with this commitment, the Ministry of Health of Ethiopia launched the Ethiopian Health Professionals Licensing Examination (EHPLE) in 2019 for undergraduates in seven core health disciplines. Since its inception, the program has expanded to encompass 17 health professions, reflecting our continued effort to build a competent and responsive health workforce.

The EHPLE was established with the primary aim of identifying and certifying health professionals who possess the minimum competencies required to provide safe and effective care. This ensures that all practicing professionals are equipped to meet the expectations of the communities they serve. The exam is coordinated by the Health and Health-Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), which consists of four specialized desks. The LEO plays a vital role in system strengthening, enabling the exam to be conducted with increased efficiency, organization, and quality assurance.

It is important to recognize that the EHPLE is not a conventional academic or recruitment examination. Rather, it is a rigorous, competency-based assessment designed to evaluate the practical skills and critical thinking abilities that are essential for professional practice. This *Practice Items Booklet* is part of our broader strategy to support examinees and educators in preparing for the licensing process. It offers a realistic overview of the exam structure, promotes a deeper understanding of its objectives, and fosters transparency and sustainability in implementation.

The development of this booklet has been made possible through the collaborative efforts of experts from higher education institutions, the Addis Ababa University Testing Center, and professional associations. Their insights and dedication have been instrumental in shaping this resource. I extend my heartfelt thanks to all contributors and to the Ministry of Health team for their tireless work and steadfast commitment to excellence in healthcare regulation.

We believe this booklet will serve as a valuable tool for candidates preparing for the examination and for educators guiding them toward success. Together, we are building a stronger, more competent health workforce for Ethiopia.

Introduction EHPLE

Welcome to the 2025 Edition of the Ethiopian Health Professionals Licensing Examination Practice Test Booklet.

This resource is designed to support candidates in their preparation for the national licensing examination, ensuring they are well-equipped with the knowledge and clinical reasoning skills required to deliver competent and ethical healthcare across Ethiopia.

In Ethiopia, the licensing examination serves as a pivotal tool in evaluating the competency of health professionals who have completed their academic training and are ready to transition into independent practice. The exam not only ensures public safety by verifying professional readiness but also upholds national and international standards in healthcare service delivery. As the healthcare system continues to expand and evolve, a strong, well-regulated professional workforce becomes even more essential.

This practice booklet contains 200 multiple-choice questions modelled after the structure, content, and format of the actual licensing examination. The questions span across core clinical domains essential for entry-level practice. This includes areas such as Patient care, leadership and management, ethics, and Health Promotion as outlined in the Information Booklet 2024.

The booklet is intended to:

- Familiarize candidates with the structure and cognitive level of the actual licensing examination.
- Offer opportunities for self-assessment and identification of strengths and gaps in knowledge.
- Encourage critical thinking and clinical decision-making in scenarios reflective of real-world healthcare practice in Ethiopia.
- Reduce exam-related anxiety by providing realistic practice experiences.

Administered under the auspices of the Ministry of Health and relevant regulatory bodies, the Ethiopian Health Professionals Licensing Examination is grounded in evidence-based frameworks and aligned with both national needs and global best practices. It has been structured to assess a range of competencies including medical knowledge, communication, professionalism, and safe clinical practice.

This practice test is not a predictive tool, and performance on it does not guarantee similar outcomes on the actual exam. However, it provides an invaluable opportunity for familiarization with question styles, pacing strategies, and content review. We strongly recommend that candidates simulate real testing conditions while using this resource, including timing their responses and reviewing rationale for both correct and incorrect answers.

The goal of licensing examinations is not only to regulate entry into the profession but also to inspire excellence and lifelong learning. By engaging with this practice test, you are taking a proactive step toward professional development, accountability, and a commitment to serving your patients with competence and compassion.

We wish you success on your licensing journey—and beyond.

Acknowledgements

This *Practice Test Items Booklet for the Ethiopian Health Professionals Licensing Examination* is the result of a collaborative effort among dedicated professionals committed to advancing the quality of health sciences education in Ethiopia. The Ministry of Health extends its sincere appreciation to all those who contributed their time, expertise, and insights to the development of this essential resource.

Special recognition is given to the item developers and reviewers from various higher education institutions—both public and private—whose academic rigor and practical experience were instrumental in shaping the content and ensuring its relevance to real-world healthcare practice. Their contributions have helped create a high-quality, standardized practice tool that reflects national competencies and global benchmarks.

We are especially grateful to the team at the Addis Ababa University Testing Center, whose technical support and academic guidance were critical throughout the process. Their leadership in assessment design and educational evaluation has significantly strengthened the quality of this booklet.

We also thank the committed staff of the Ministry of Health, particularly those within the Health and Health-Related Institutions and Professionals' Regulatory Lead Executive Office (HHRIPR LEOs), whose coordination, oversight, and unwavering dedication ensured the successful completion of this project.

Together, these collective efforts reflect our shared vision of building a competent, ethical, and resilient health workforce for Ethiopia.

Medicine

1. A 60-year-old male patient underwent surgery for a perforated peptic ulcer disease. Subsequently, he developed paralytic ileus and frequently complained of tingling and numbness. He received treatment for hypocalcemia. Serum electrolytes showed an ionized serum calcium level of 4.2 mg/dL, a magnesium level of 1 mg/dL, and a potassium level of 3.5 mg/dL.

What is the most likely explanation for this patient's condition?

- A. Hypocalcemia
- B. Hypokalemia
- C. Hypomagnesemia
- D. Hyperphosphatemia

ANSWER: C

2. A 30-year-old male soldier, who sustained a vascular injury, was transfused with six units of blood. The bleeding was arrested at the major OR. However, complained of dyspnea and cough two hours after the transfusion. Physical examination showed bilateral rales at the base of the lung, and JVP was raised.

What is the most likely diagnosis of this patient?

- A. Blunt lung injury
- B. Cardiac tamponade
- C. Transfusion reaction
- D. Pulmonary edema

ANSWER: D

3. A 30-year-old male patient sustained a stab injury to his left chest three hours ago. On arrival, he was in respiratory distress, and there was a 2cm by 2cm stab wound at the third posterior intercostal space. He had decreased air entry and dullness over the lower half of the left hemithorax.

What is the most appropriate site of chest tube insertion for this patient?

- A. Fourth intercostal space along anterior axillary line
- B. Fifth intercostal space along anterior axillary line
- C. Sixth intercostal space along mid-axillary line
- D. Fifth intercostal space at mid-axillary line

ANSWER: D

4. A two-year-old female child was brought to an emergency room with a complaint of right leg pain and inability to walk, which had persisted for one day. She has an associated high-grade fever and chills. On examination, her pulse rate was 148/min and her temperature was 38.6 degrees Celsius. Her WBC count was 20,000 cells/dL.

What is the best diagnostic modality for this case?

- A. Ultrasound
- B. Leg x-ray
- C. CT-scan
- D. MRI

ANSWER: D

5. A 45-year-old male patient presented to an emergency OPD after he sustained a fall from a four-meter-high building. On evaluation, his vital signs were within the normal range, he had a deformed right femur with angulations and shortening, but there was no laceration. He is going to be referred to a nearby hospital for an x-ray.

What is the most appropriate management for this patient before referral?

- A. Long leg posterior gutter
- B. Antibiotics and analgesics
- C. Splinting his leg with a circular cast
- D. IV fluid resuscitation and TAT

ANSWER: A

6. A 30-year-old female patient came to a surgical OPD with complaints of breast swelling and pain of two months duration. On evaluation, she had a well-circumscribed, mobile, 5cm by 6cm breast mass. On investigation, ultrasound suggested a benign mass, and FNAC showed a giant fibroadenoma with atypia.

What is the best management for this patient?

- A. Repeat FNAC
- B. Incisional biopsy
- C. Excisional biopsy
- D. Follow-up ultrasound biannually

ANSWER: C

7. A 45-year-old female patient presented to a surgical OPD with right breast swelling of two years duration. She also had pain and swelling over her axillary region. On evaluation, she had a fungating mass on her right breast that involves the nipple-areola complex and palpable enlarged axillary lymphadenopathy.

What is the most likely diagnosis of this patient?

- A. Invasive ductal carcinoma
- B. Invasive lobular carcinoma
- C. Inflammatory breast cancer
- D. Paget's disease of the breast

ANSWER: A

8. A 60-year-old female patient presented to a surgical OPD with a complaint of anterior neck swelling of three years duration. The swelling was initially small but gradually increased in size. She also had associated heat intolerance, irritability, and sleeplessness. On evaluation, the pulse rate was 120/min, and both left and right lobes were enlarged.

What is the most likely diagnosis of this patient?

- A. Grave's disease
- B. Toxic adenoma
- C. Simple multinodular goiter
- D. Toxic multinodular goiter

ANSWER: D

9. A six-year-old male child, who had complaints of cough and fever for the last two weeks, was admitted to a pediatric ward. On evaluation, he was in cardiorespiratory distress, his pulse rate was 140/min and respiratory rate was 38/min. There were bronchial breath sounds as well as decreased air entry in the right lung field. A chest x-ray showed hydrothorax. Ultrasound suggested loculated empyema.

What is the most appropriate initial management for this patient?

- A. Fibrinolytic administration
- B. Needle aspiration and IV antibiotics
- C. Chest tube insertion
- D. Open thoracotomy

ANSWER: C

10. A 50-year-old female patient presented to a surgical OPD with a complaint of progressive difficulty of swallowing of the last four months. She had associated significant weight loss, fatigue, and loss of appetite. Barium swallow showed abrupt tapering with shouldering and mucosal irregularity.

What is the most likely diagnosis of this patient?

- A. Achalasia
- B. Schiatzki's ring
- C. Esophageal cancer
- D. Diffuse esophageal spasm

ANSWER: C

11. A 60-year-old male patient presented to a medical OPD with chest pain and hemoptysis of three months duration. Assessment findings showed malnutrition. A chest x-ray revealed a 4cm by 5cm eccentric lesion on the right lung with pleural effusion.

What is the most likely diagnosis of this patient?

- A. Lung cancer
- B. Aspergilloma
- C. Tuberculosis
- D. Solitary pulmonary nodule

ANSWER: A

12. A 56-year-old male patient came to a hospital and complained of severe crampy pain in his right leg for the last three months. The pain was worsened by walking and leg elevation and was relieved by taking rest. On examination, his blood pressure is 155/60 mmHg, he has shiny skin and hypertrophied nails, there is no visible hair on his right leg, and sensation is equal on both legs. On the right side, dorsalis pedis, posterior tibialis, and popliteal pulses were not palpable.

What is the most appropriate initial step of workup for this patient?

- A. Doppler ultrasound
- B. Ankle brachial index
- C. Angiography
- D. Abdomino-pelvic CT

ANSWER: A

13. A 60-year-old male patient who lives in a rural part of Ethiopia presented to an emergency OPD with a complaint of failure to pass feces and flatus for one day. In addition, he had significant abdominal distention, nausea, infrequent vomiting, and abdominal pain. Otherwise, there was no history of weight loss or bowel habit change. His physical examination showed a blood pressure of 110/70 mmHg, a heart rate of 98/min, and a protuberant abdomen. On digital rectal examination, an empty and ballooned-out rectum with no blood on the examining finger was noted.

What is the most likely diagnosis of this patient?

- A. Primary small bowel volvulus
- B. Secondary small bowel volvulus
- C. Sigmoid volvulus
- D. Colorectal cancer

ANSWER: C

14. A 40-year-old male patient presented to an emergency OPD with a complaint of failure to pass feces and flatus for two days. Associated with this, he had abdominal distention, nausea, frequent vomiting, and a high-grade fever. On physical examination, his abdomen was tender and rigid; his blood pressure and heart rate were 90/60 mmHg and 110/min, respectively.

What is the most appropriate initial management for this patient?

- A. Laparotomy
- B. IV fluid resuscitation
- C. Broad-spectrum antibiotics
- D. Adrenaline drip

ANSWER: B

15. A 30-year-old female patient, who has a history of treatment for peptic ulcer disease, came to an emergency OPD with a complaint of sudden onset of profuse bloody vomiting of 10 hours duration. She also has easy fatigability, syncope, and melena. However, she did not have history of dysphagia, jaundice, weight loss, or bowel habit change.

What is the most likely explanation for this patient's case?

- A. Bleeding esophageal varices
- B. Bleeding gastric cancer
- C. Bleeding duodenal ulcer
- D. Lower GI bleeding

ANSWER: C

16. A 13-year-old male child presented to a surgical OPD with abdominal pain and anorexia of two days duration. The pain was initially at the periumbilical area, which later shifted to the right lower quadrant. He also has a low-grade intermittent fever. On examination, his pulse rate is 110/min, and he has direct and rebound tenderness at the right lower quadrant.

What is the most likely diagnosis of this child?

- A. Terminal ileitis
- B. Pyelonephritis
- C. Chron's disease
- D. Acute appendicitis

ANSWER: D

17. A 40-year-old female patient came to a surgical OPD with a complaint of epigastric burning pain in the last three weeks. She reported that her father died of gastric cancer at the age of 75. On investigation, upper GI endoscopy was normal, and urease test was positive. The physician reassured her, but she asked if there is any preventive measure to reduce the risk of gastric cancer.

What is the most appropriate advice for this patient?

- A. Eating smoked meats
- B. Eating fewer vegetables
- C. Eating high-salt foods
- D. H. pylori eradication therapy

ANSWER: D

18. A 70-year-old man came to a surgical clinic with a complaint of right upper quadrant abdominal pain of two years duration. He has no fever or vomiting, but he has a repeated history of admission for similar complaints. The findings from his physical examination are unremarkable. Abdominal ultrasound reveals multiple echogenic foci, and a fibrosed and shrunken gallbladder.

What is the most likely explanation for this finding?

- A. Acute cholecystitis
- B. Chronic cholecystitis
- C. Hydropic gallbladder
- D. Gallstone ileus

ANSWER: B

19. A 43-year-old female patient complained of right upper quadrant abdominal pain that radiated to the back of eight hours duration. She had a similar complaint for the last year and she was given pantoprazole but did not show significant improvement. Abdominal ultrasound showed multiple gallstones and a shrunken gallbladder.

What is the most likely diagnosis of this patient?

- A. Acute gastritis
- B. Acute cholecystitis
- C. Symptomatic cholelithiasis
- D. Gastroesophageal reflux disease

ANSWER: C

20. A 30-year-old woman, who is a known RVI patient on HAART, came to an emergency OPD with complaints of severe perianal pain, fever, chills, and rigors of two days duration. On further evaluation, a tender fluctuant mass over the right perianal area was noticed.

What is the most appropriate management for this patient?

- A. IV antibiotics and fistulotomy
- B. IV antibiotics, incision, and drainage
- C. IV antibiotics and excision
- D. IV antibiotics only

ANSWER: B

21. A 67-year-old male patient came to a clinic and complained of right flank pain of three days duration, which is associated with three episodes of non-projectile vomiting. He was given IV antibiotics for three days, but no improvement was seen. On evaluation, his pulse rate is 112/min, respiratory rate is 22/min, blood pressure is 90/60 mmHg, temperature is 38.0 degrees Celsius, and he has right flank costovertebral angle tenderness.

What is the most appropriate next step in this patient's care?

- A. Abdominal CT
- B. Ultrasound
- C. KUB
- D. IVP

ANSWER: A

22. A 60-year-old male patient came to a regular OPD with a complaint of difficulty of urination in the last three weeks. His urinalysis result showed many RBCs in high-power field. An abdominopelvic ultrasound revealed a thickened urinary bladder at the trigone.

What is the most appropriate next step in this patient's care?

- A. KUB
- B. Cystoscopy
- C. Pelvic MRI
- D. Pelvic CT scan

ANSWER: B

23. A 55-year-old male patient was referred from a local clinic to a hospital with a diagnosis of testicular tuberculosis after he presented with testicular swelling of five months duration. He had significant weight loss, night sweats, and a low-grade intermittent fever. For this complaint he was given anti-TB for six months, but no improvement was seen. On evaluation, there was a 4cm by 6cm, non-tender, and hard mass at the left testis.

What is the most appropriate next step in the care of this patient?

- A. FNAC and continue anti-TB
- B. FNAC and consider MDR TB
- C. Inguinal orchiectomy and biopsy
- D. Inscional biopsy and continue anti-TB

ANSWER: C

24. A three-day-old neonate was presented to a NICU with frequent bilious vomiting of six hours duration. On examination, the neonate is dehydrated, and its abdomen is distended. An abdominal x-ray showed multiple air-fluid levels.

What is the most appropriate initial step in the management of this neonate?

- A. Laparotomy
- B. Fluid resuscitation
- C. Perform upper GI series
- D. Start him on IV antibiotics

ANSWER: B

25. A 27-year-old primigravida came to an antenatal clinic for routine care at a gestational age of 35th week from reliable LNMP. She reported that she had urinary frequency and nocturia. Her vital signs were within the normal range. An abdominopelvic ultrasound showed an increased size of both kidneys by 1-2cm and mild hydronephrosis. Urine analysis revealed no abnormality. What is the most likely explanation for this finding?
- A. Urinary tract infection
 - B. Physiologic change of pregnancy
 - C. Acute kidney injury
 - D. Urinary tract obstruction
- ANSWER: B
26. A 42-year-old G-V, P-IV (3-alive) mother came to the ANC clinic for routine care. Obstetric ultrasound showed a normal twin pregnancy with AGA of 20 weeks. Her previous pregnancy was complicated by raised blood pressure, resulting in stillbirth delivery. The physician advised her to have strict ANC follow-up, as she is at risk of developing high blood pressure during this pregnancy. What is the most likely predisposing factor in this case?
- A. Twin pregnancy
 - B. Previous raised blood pressure
 - C. Multigravidity
 - D. Old age
- ANSWER: B
27. A 25-year-old G-II, P-I mother, with a gestational age of 38-week from reliable LNMP, came to an antenatal clinic with complaints of dark red vaginal bleeding and abdominal pain of three hours duration. Her blood pressure is 155/110 mmHg and pulse rate is 120/min. On abdominal examination, there is tenderness over the uterus, and FHB is 146/min. What is the most likely diagnosis of this patient?
- A. Placenta previa
 - B. Placenta abruption
 - C. Uterine rupture
 - D. Vasa previa
- ANSWER: B
28. A 35-year-old G-VI, P-III, A-II woman, with a gestational age of 22-week, came to the ANC clinic for routine care. She had a history of exertional dyspnea, PND, and orthopnea, which started before the current pregnancy. On physical examination, her vital signs were within the normal range, and she had a Grade-III holosystolic murmur at the 5th intercostal space radiating to the axilla. Her laboratory tests showed normal findings. The physician sent her for radiologic evaluation. What is the most appropriate radiographic investigation to assess this patient's condition?
- A. Chest X-ray
 - B. Electrocardiography
 - C. Echocardiography
 - D. CT scan
- ANSWER: C

29. A 22-year-old primigravida at a gestational age of 40-week came to a labor and delivery ward with pushing down pain of 12 hours duration. She has no gush of fluid per vagina. Her ANC follow-up was uneventful. On physical examination, her vital signs are within the normal range, uterine contractions are 3 over 10 minutes lasting for 30s to 35s, and there is a normal FHB pattern. On pelvic examination, the cervix is closed. On evaluation, four hours after pethidine 50mg IM administration, there is no cervical change.

What is the most likely diagnosis of this woman?

- A. Prolonged latent first stage labor
- B. False labor
- C. Lightening
- D. Arrest of cervical dilatation

ANSWER: B

30. A 29-year-old G-II, P-1 (alive) mother at a gestational age of 34-week came to a labor and delivery ward with pushing down pain of 12 hours duration. She has a severe global headache and blurring of vision of six hours duration. On physical examination, her blood pressure is 165/105 mmHg, uterine contractions are 3 over 10 minutes lasting for 45s and there is a normal FHB pattern. On pelvic examination, the membrane is ruptured and clear, the cervix is fully dilated, and the station is at +2cm.

What is the most appropriate management for this mother?

- A. Shortening the second stage of labor by vacuum delivery
- B. Immediate delivery by cesarean section
- C. Shortening the second stage of labor by forceps delivery
- D. Admitting to the labor ward and giving more time

ANSWER: C

31. A 25-year-old P-I mother came to a postnatal clinic after giving birth to a live male neonate at a nearby health center two months ago. She is breastfeeding her baby and has not seen menses since delivery. She asked her physician whether she should use contraception.

What is the most appropriate advice for this mother?

- A. There is no need for contraception use because she is amenorrheic
- B. She has to start contraception use to prevent pregnancy
- C. She has to avoid contraception use because she is breastfeeding
- D. Abstinence from sexual intercourse until she intends to have her next pregnancy

ANSWER: B

32. A 37-year-old G-VI, P-V (all alive) woman with a gestational age of 33-week from reliable LNMP came to a labor and delivery room with pushing down pain of six hours and rupture of the membrane of eight hours duration. Her last three deliveries were complicated by preterm births. On examination, her vital signs are within the normal range, uterus is 38-week sized, and cervix is 3 cm dilated and 60 percent effaced. Obstetric ultrasound evaluation shows normal twin pregnancy.

What is the major predisposing risk factor for preterm delivery for this mother?

- A. Twin pregnancy
- B. PROM
- C. Prior preterm deliveries
- D. Multiparity

ANSWER: C

33. A 32-year-old G-III, P-II (all alive) woman, who has been amenorrheic for the last four months, presented with anterior neck swelling of two years duration, associated with palpitation and easy fatigability. On physical examination, she is tachycardic, and there is a 4 cm by 4 cm anterior neck mass which moves with swallowing.

What are the most likely laboratory test results that explain the patient's condition?

- A. Low TSH and low free T4
- B. High TSH and low free T4
- C. Low TSH and high free T4
- D. High TSH and high free T4

ANSWER: C

34. A 26-year-old mother was referred to an obstetric emergency unit at the 10th week of gestation with a complaint of profuse vaginal bleeding with passage of liver-like fleshy material for the past three hours. On examination, her blood pressure is 90/60 mmHg, pulse rate is 110/min, and digital pelvic examination reveals active bleeding with tissue palpable per cervical os.

What is the most appropriate management for this patient?

- A. Provide her with sublingual misoprostol
- B. Provide her with oral misoprostol
- C. Do manual vacuum aspiration
- D. Wait for spontaneous expulsion

ANSWER: C

35. A 29-year-old G-II, A-I woman came to a health care unit with lower abdominal pain of two days duration which is associated with vaginal spotting. She has been amenorrheic for the past two months. On investigation urine hCG is positive and ultrasound shows an empty uterus, free peritoneal fluid, and a 4cm by 5cm right adnexal echo complex mass.

What is the most likely diagnosis of this patient?

- A. Ectopic pregnancy
- B. Appendiceal mass
- C. Complete abortion
- D. Ovarian torsion

ANSWER: A

36. A 19-year-old primigravida came to a hospital with frequent vomiting of two weeks duration. Gestational age from LNMP is 7 weeks and 5 days. She is passing feces and flatus and she has no history of dysuria or urine color change. Laboratory results show that urine ketone is +3 and RBS is 105mg/dl.

What is the most likely diagnosis of this case?

- A. Hyperemesis gravidarum
- B. Diabetic ketoacidosis
- C. Bowel obstruction
- D. Peptic ulcer disease

ANSWER: A

37. A 27-year-old multigravida mother came to a labor and delivery ward with a complaint of pushing down pain following a gush of fluid per vagina. The gestational age from reliable LNMP is 31 weeks and 3 days. She eats three to four times per day. She had a history of twin delivery, and both of them were male.

What is the most likely cause for labor in this scenario?

- A. Premature rupture of membranes
- B. Unexplained preterm labor
- C. Indicated preterm labor
- D. Higher-order gestation

ANSWER: A

38. A 21-year-old primigravida came to the ANC clinic for routine care. The gestational age from an early ultrasound which was done at the 9th week is 42 weeks and 6 days. The current ultrasound examination shows gestational age of 39-week, EFW of 3750g, fundal placenta, and no measurable amniotic fluid.

For what complication is this pregnancy at greatest risk?

- A. Meconium aspiration
- B. Cephalopelvic disproportion
- C. Antepartum haemorrhage
- D. Cord prolapses

ANSWER: A

39. A 19-year-old primigravida whose gestational age is 32-week came to a hospital with three episodes of generalized abnormal body movement characterized by up-ward rolling of the eyes and drooling of saliva in the last three hours. She has no pushing down pain. On examination, her GCS is 15/15 and her blood pressure is 140/100mmHg. On abdominal examination, FHB is positive and she has no uterine contraction.

What is the definitive management for this patient?

- A. Securing IV lines, initiating MgSO₄ and IV fluids
- B. Admission to maternity ward and initiation of Dexamethasone
- C. Termination of pregnancy
- D. Management of hypertension and planning for outpatient management

ANSWER: C

40. A 32-year-old nulliparous woman came to a clinic with malodorous vaginal discharge that is associated with lower abdominal pain. On examination, her pulse rate is 110/min, axillary temperature is 37.9 degrees Celsius, and there is a cervical motion and adnexal tenderness.

What is the most likely long-term complication if the patient is not treated appropriately?

- A. Disseminated gonococci syndrome
- B. Tubo-ovarian abscess
- C. Perihepatitis
- D. Ectopic pregnancy

ANSWER: D

41. A 54-year-old postmenopausal woman came to a gynecologic OPD with mass protrusion per vagina of three years duration. The mass is irreducible and restricts her from doing daily routine activities. On pelvic examination, there is a fleshy mass outside the introitus. Anterior and posterior vaginal walls have no defect.

What is the most appropriate management for this woman?

- A. Placing pessary
- B. Hysterectomy
- C. Kegel's exercise
- D. Reduction of prolapse

ANSWER: B

42. A 20-year-old multiparous woman came to an emergency OPD with abnormal menstrual bleeding of four months duration. On speculum examination, the cervix appears normal. Her hemoglobin is 11.2g/dl, urine-hCG is negative, and ultrasound evaluation reveals normal results.

What is the most appropriate management to stop the bleeding in this patient?

- A. Dilatation and Curettage
- B. Hysterectomy
- C. High dose IV oxytocin
- D. Combined oral contraceptive

ANSWER: D

43. A 37-year-old P-V woman came to a gynecologic OPD with a complaint of failure to see menses for one and a half years. On evaluation, her blood pressure is 120/70mmHg, pulse rate is 82/min, and abdominal examination is unremarkable. Genital examination shows an atrophied vulva. Ultrasound shows a normal uterus and free adnexa.

What is the most likely diagnosis of this patient?

- A. Premature menopause
- B. Physiologic menopause
- C. Lactational amenorrhea
- D. Artificial menopause

ANSWER: A

44. A 21-year-old woman has been highly concerned about cervical cancer prevention ever since her mother passed away while on treatment with radiotherapy for cervical cancer. She started coitus at the age of 14-year and she sometimes uses post pill. Three years back, she had a history of medical abortion which was complicated with anemia and sepsis.

What is the most likely predisposing factor for this condition?

- A. Age at first coitus
- B. Her current age
- C. History of abortion
- D. History of post-pill

ANSWER: A

45. A 30-year-old P-II, A-I, GTD-I woman came to a gynecologic clinic with a complaint of vaginal bleeding of one month duration. She also has cough, chest pain, and hemoptysis. She has a history of suction curettage one year back. On investigation, serum hCG is 20,000 mIU/ml, and an ultrasound scan shows a 4cm by 6cm intrauterine mass.

What is the most likely diagnosis of this patient?

- A. Endometrial cancer
- B. Complete mole
- C. Choriocarcinoma
- D. Uterine sarcoma

ANSWER: C

46. A 36-year-old multiparous woman came to a gynecologic clinic for a family planning service. She is a known hypertensive patient on medication. She also reported that she smokes cigarettes occasionally. On examination, she is well-looking; her blood pressure and BMI are 150/100 mmHg and 27.8 kg/m², respectively. A week ago, she noticed left breast swelling which is fixed with the chest wall and a biopsy confirmed that it is breast cancer.

What is the most appropriate contraceptive for this woman?

- A. Intrauterine contraception device
- B. Combination oral contraception
- C. Progesterone-only pills
- D. Barrier method

ANSWER: D

47. A 33-year-old P-V woman came to a hospital for a family planning service. She preferred long-acting contraceptives over short-acting, and an implant was inserted over her left arm. Two months later, she came back with a complaint of irregular vaginal bleeding. On examination, her conjunctiva is pink, her chest is clear, and there is no murmur or gallop. Investigation results show a hematocrit level of 45 percent and unremarkable pelvic ultrasound.

What is the most appropriate management for this patient?

- A. Provide her with ferrous sulfate
- B. Remove implants
- C. Provide COC
- D. Reassure her

ANSWER: D

48. An 11-year-old child was brought to a gynecologic clinic accompanied by a policeman. She reported that she had experienced sexual assault seven days ago while coming from school to home. She also informed that she had developed significant bleeding following this incident. It was further learned that she had stayed with the perpetrator for three days during which he had sexual intercourse multiple times. The serostatus of the perpetrator is not known. Her general physical examination findings are unremarkable.

What is the most appropriate next step in the evaluation of this patient?

- A. Investigate serostatus
- B. Do a genital examination
- C. Send for X-ray
- D. Do urine hCG

ANSWER: B

49. A three-year-old male child was brought to a pediatric OPD with a complaint of failure to sit unsupported. The mother did not have ANC follow-up and the delivery was at home. On history, the child did not cry immediately after delivery. On examination, anthropometry was affected, and he could not sit unsupported. Motor examination revealed that tone was hypertonic, and deep tendon reflex was 4/4 in the lower extremity bilaterally.

What is the most likely cause of the child's developmental abnormality?

- A. Cerebral palsy
- B. Hypothyroidism
- C. Hypotonia syndrome
- D. Neuromuscular diseases

ANSWER: A

50. A 12-month-old male infant was presented to a pediatric OPD with a complaint of watery diarrhea of one week duration. He also had swelling of legs, high-grade fever, and non-productive cough. His mother reported that he was weaned and started on cow's milk at the age of eight months due to her recent pregnancy. On physical examination, he was conscious and alert, weight for age was less than - 2 SD, MUAC was 12cm, and there was corneal opacity and conjunctiva clouding. Chest examination revealed mild subcostal and intercostal retraction and coarse crepitation over the right lower 1/3rd of the posterior lung field. There was bilateral Grade-III pitting pedal edema.

What is the most likely diagnosis of this case?

- A. Severe wasting plus conjunctivitis
- B. Edematous malnutrition plus Vitamin A deficiency
- C. Congestive heart failure plus conjunctivitis
- D. Nephritic syndrome plus Vitamin A deficiency

ANSWER: B

51. A two-month-old male infant was brought to a pediatric emergency with complaints of shortness of breath and hoarseness of voice after he took the first vaccine. On physical examination, his pulse rate is 168/min, respiratory rate is 62/min, and axillary body temperature is 38.6 degrees Celsius. He also has inspiratory stridor and a diffuse papular rash all over the body.

What vaccine is most likely contraindicated for this child on the next visit?

- A. DPT vaccine
- B. ROTA vaccine
- C. OPV vaccine
- D. IPV vaccine

ANSWER: A

52. A three-year-old male child was brought to a pediatric emergency with a complaint of cough of five days duration. He also has high-grade intermittent fever, shortness of breath, fast breathing, and chest pain. On physical examination, his pulse rate is 148/min, respiratory rate is 58/min, and temperature is 39.5 degrees Celsius. Besides, he has subcostal and intercostal retraction, stony dullness, and absent air entry over the right posterior lower 2/3rd of the lung field.

What is the most important investigation for this child?

- A. Lumbar puncture
- B. Paracentesis
- C. Thoracentesis
- D. Pericardiocentesis

ANSWER: C

53. A 20-minute-old male neonate was referred to a NICU from the maternity ward due to a complaint of failure to initiate and sustain breathing at birth. The delivery was via cesarean section at the gestational age of 40-week for an indication of Grade-III meconium-stained amniotic fluid. The neonate had APGAR scores of 1, 3, and 5 at 1st, 5th, and 10th minutes respectively. On examination, his pulse rate is 174/min, respiratory rate is 28/min gasping type and axillary body temperature is 35.7 degrees Celsius. On chest examination, he has nasal flaring, and severe subcostal and intercostal retraction with coarse crackles over the right posterior 1/3rd of the lung field. On neurologic examination, he was comatose and had flaccid muscle tone, deep tendon reflexes were absent, and pupils were unequal with poor light reflex.

What is the most likely cause of this neonate's clinical presentation?

- A. Cyanotic congenital heart disease with deoxygenated blood in the systemic circulation
- B. Severe anemia secondary to severe hemorrhage or hemolytic disease
- C. Shock severe enough to interfere with the transport of oxygen
- D. Failure to breathe after birth because of in-utero CNS injury

ANSWER: D

54. A 20-minute-old female neonate was brought to a NICU due to the complaint of failure to initiate and sustain breathing at birth. The delivery was via emergency cesarean section at the gestational age of 41-week for an indication of Grade-III meconium-stained amniotic fluid. The neonate had APGAR scores of 7 and 8 at the 1st and 5th minutes respectively. On examination, her pulse rate was 164/min, respiratory rate was 68/min and axillary body temperature was 37.7 degrees Celsius, and she had nasal flaring, severe subcostal-intercostal retraction, and coarse crackles over the right posterior 1/3rd of the lung field. The neonatal reflexes were normal and the tone was normotonic. Chest x-ray showed patchy infiltrates, coarse streaking of both lung fields with flattening of the diaphragm, and increased anteroposterior diameter of the chest.

What is the most appropriate management option for the neonate?

- A. Routine antibiotic therapy and calcium
- B. Extracorporeal membrane oxygenation and Epinephrine
- C. Supportive care and exogenous surfactant
- D. Supportive care and management of respiratory distress

ANSWER: D

55. A two-hour-old male newborn was transferred to a Neonatal ICU for general evaluation. He was delivered after 10 hours of labor through spontaneous vaginal delivery at 41-week of gestation. On physical evaluation, the birth weight was 2000g which makes the birth weight for gestational age less than the 10th percentile.

What is the most likely complication that could happen in this neonate?

- A. Anemia
- B. Birth trauma
- C. Hypoglycemia
- D. Hypothyroidism

ANSWER: C

56. A two-hour-old male term neonate was brought to a Neonatal ICU with a complaint of respiratory distress since birth. The delivery was by spontaneous vaginal delivery after one hour of labor. The APGAR scores at the 1st and 5th minutes were 8 and 9, respectively. Chest radiograph shows perihilar streaking and fluid in interlobar fissures.

What is the most likely diagnosis of the neonate?

- A. Spontaneous pneumothorax
- B. Respiratory distress syndrome
- C. Meconium aspiration syndromes
- D. Transient tachypnea of the newborn

ANSWER: D

57. A one-year-old unvaccinated male infant was brought to an emergency department with a complaint of high-grade fever of two days duration. Associated with this, he had a non-productive type of cough and skin rash. On examination, axillary body temperature is 39.7 degrees Celsius, and he has macular lesions on the buccal mucosa opposite to the lower premolars, hyperemic conjunctiva, no retraction, clear chest with good air entry bilaterally, and diffuse maculopapular rash all over the body.

What is the most appropriate preventive measure needed for those individuals who have exposure to this infant?

- A. Immunoglobulin after 6 days of exposure
- B. Measles vaccine within 72 hours of exposure
- C. Avoid exposure for 4-6 days after the onset of rash
- D. Give vaccine and immunoglobulin at the same time

ANSWER: B

58. An 11-month-old unvaccinated male child was brought to an emergency department with a complaint of difficulty of sucking and opening his mouth, generalized muscle rigidity, and spasm. Two weeks before the onset of the current complaint, he sustained an injury while crawling. The wound is clean over the knee area. He is conscious and alert.

What is the most appropriate management for this child?

- A. Tracheotomy
- B. Tetanus antitoxoid (TAT)
- C. 3rd generation cephalosporin
- D. Wound excision and debridement

ANSWER: B

59. An 18-month-old male child was brought to an emergency OPD with a complaint of cough for three weeks. He also has recurrent episodes of diarrhea and failure to thrive. On physical examination, the child's MUAC was 10.5cm, had extensive oral thrush, lymphadenopathy, and hepato-splenomegaly. A rapid HIV antibody test was found to be positive.

What is the most appropriate first-line ART regimen for this child?

- A. AZT + 3TC + NVP
- B. ABC + 3TC + NVP
- C. AZT + 3TC + LPV/r
- D. ABC + 3TC + LPV/r

ANSWER: C

60. A seven-year-old female child was brought to a pediatric OPD with complaints of bloody diarrhea, abdominal cramps, and step-ladder fever. On physical examination, there was hepato-splenomegaly. CBC result showed low leukocyte count and blood culture grew Gram-negative bacilli.

What is the most appropriate preventive measure that minimizes the transmission of the disease to the community?

- A. Hand hygiene
- B. Antibiotic prophylaxis
- C. Vaccine for the community
- D. Care in handling of dead animals

ANSWER: A

61. A six-month-old male infant with complaints of cough, tachypnea, and audible wheezing was admitted to a hospital. His older sibling has also developed a minor respiratory illness over the past week. Physical examination of the infant indicated that he had tachypnea, wheezing, and crackles. Complete blood count showed normal white blood cell and differential counts.

What is the most likely finding on the chest x-ray of this infant?

- A. Atelectasis
- B. Lobar opacity
- C. Pleural effusion
- D. Hilar lymphadenopathy

ANSWER: A

62. A one-year-old male child was presented to an emergency department with complaints of cough and fever of seven days duration. On physical examination, he had signs of severe respiratory distress, and he was tachypneic. Besides, stony dullness and decreased air entry were noted over the right chest. Needle aspiration of the right chest revealed clear pus.

What is the most appropriate management for this child?

- A. Sclerosing agent
- B. Chest tube drainage
- C. Wait for the response to antibiotics
- D. Thoracoscopic surgery

ANSWER: B

63. A seven-year-old male child was brought to a pediatric OPD with a complaint of migratory joint pain, which involved both the upper and lower extremities, and associated high-grade fever. He recalls a recent history of sore throat, which improved without any treatment. On physical examination, his pulse rate is 114/min regular, his axillary body temperature is 39.1 degrees Celsius, and there is active precordium with a Grade-IV holosystolic murmur at the apex, which radiates to the axilla. Blood tests revealed an ESR of 54 mm/hr and the ASO titer is positive. Chest x-ray shows cardiomegaly.

What is the most likely diagnosis of this patient?

- A. Acute rheumatic fever
- B. Rheumatic recurrence
- C. Infective endocarditis
- D. Myocarditis

ANSWER: A

64. An eight-year-old male child was diagnosed with rheumatic heart disease a year ago. Currently, he complains of a high-grade intermittent fever and associated reddish discoloration of the urine for one week. On physical examination, an axillary body temperature of 39.5 degrees Celsius, a pulse rate of 110/min, an active precordium, a Grade-IV holosystolic murmur, and hepatosplenomegaly were identified. From his blood test, mild normocytic anemia and ESR of 174 mm/hr, and blood +4 and many RBCs/HPF on urinalysis were noted. Transthoracic echocardiography showed large vegetation over the mitral valve leaflet.

What is the most appropriate management option for this child?

- A. Vancomycin and beta-lactam antibiotic for 6 weeks
- B. Beta-lactam antibiotic and cloxacillin for 6 weeks
- C. Beta-lactam antibiotic and gentamicin for 6 weeks
- D. Vancomycin and cloxacillin for 6 weeks

ANSWER: C

65. A seven-year-old male child came to a health center with complaints of watery diarrhea, which progressed to grossly bloody and was associated with abdominal pain and discomfort. He also complained of flatulence and mucus in stools. Laboratory examination of the stool revealed heme-positive stools but no fecal leukocytes.

What is the most likely diagnosis of this case?

- A. Amebic colitis
- B. Ulcerative colitis
- C. Giardiasis
- D. Dysentery

ANSWER: A

66. A four-year-old male child was brought to a hospital with complaints of yellowish discoloration and abdominal pain of one week duration. He had a history of blood transfusion before six weeks. A physician considered viral hepatitis infection and sent a serologic test for hepatitis B virus.

What serologic test will show active acute infection in this child?

- A. HBeAg
- B. HBcAg
- C. HBsAg
- D. HBV DNA

ANSWER: A

67. A three-year-old female child was brought to a pediatric emergency with a complaint of reddish discoloration of urine of two days duration. She also has increased frequency of urination, pain during urination, and urgency. On physical examination, axillary body temperature is 37.6 degrees Celsius, and she has suprapubic tenderness. Urine analysis showed blood +2, and urine microscopy revealed 6-8 WBCs/HPF.

What is the most likely diagnosis of this child?

- A. Urolithiasis
- B. Pyelonephritis
- C. Cystitis
- D. Glomerulonephritis

ANSWER: C

68. A one-month-old male infant was brought to follow-up clinic for evaluation of persistent jaundice after two weeks of life. His mother reported that he has feeding difficulty and excessive sleepiness. On examination, he is sluggish and hypothermic, and has a puffy face, low-set ears, up-slanting palpebral fissures and icteric sclera. An umbilical hernia was noted on the abdominal exam.

What is the most likely expected laboratory abnormality in this patient?

- A. Low TSH and low T4
- B. High TSH and low T4
- C. Low TSH and normal T3
- D. High TSH and high T3

ANSWER: B

69. A two-year-old male child was brought to a hospital with complaints of high-grade fever and projectile vomiting of ingested matter for two days. From neurologic examination, it was noticed that the child has neck stiffness, and Brudzinski and Kernig signs were also positive. On laboratory investigation, a WBC count of 1000 cells with 80 percent neutrophils and 20 percent lymphocytes, glucose of 30 gm/dl, protein of 150 mg/dl, gram-positive lancet-shaped diplococci in chains on Gram stain were noted.

What is the most likely etiologic agent in this case?

- A. Pneumococcus
- B. Meningococcus
- C. H. influenza
- D. Staphylococcus aureus

ANSWER: A

70. A nine-month-old female infant was brought to a hospital with a complaint of irritability of one month duration. Her mother also complained that she has not yet started to sit unsupported. Her mother was a strict vegetarian for the last year. Investigation revealed a hemoglobin level of 8 gm/dl with an MCV of 120 fl, and peripheral morphology showed hypersegmented neutrophils.

What is the most likely diagnosis of this infant?

- A. Folate deficiency
- B. Vitamin B12 deficiency
- C. Aplastic anemia
- D. Iron deficiency anemia

ANSWER: B

71. A 10-year-old male child presented to an OPD with complaints of protrusion of the eye, eye pain, dropping of the eyelid and visual impairment of one month duration. Biopsy of the mass showed small, round, blue cell tumors.

What is the most likely diagnosis of this child?

- A. Retinoblastoma
- B. Rhabdomyosarcoma
- C. Neuroblastoma
- D. Periorbital cellulitis

ANSWER: B

72. A five-year-old male child was brought to an emergency OPD with a complaint of bilateral weakness of two days duration. Initially, the child was complaining of numbness in the hands and feet and then he experienced a "heavy," weak feeling in the legs, which was followed by an inability to walk. The weakness ascends to involve the arms, trunk, throat, and face. On physical examination, deep tendon reflexes are absent with no sensory level.
What is the most likely diagnosis of this child?
- A. Acute spinal cord syndrome
 - B. Guillain-Barre syndrome
 - C. Transverse myelitis
 - D. Tick paralysis
- ANSWER: B
73. A five-year-old male child had developed a fever and right ear pussy discharge of one week duration. Otoscopic examination showed frank pus coming out of the right ear canal and central right tympanic membrane perforation. An ear discharge sample was sent for bacteriologic analysis.
What is the most likely microorganism responsible for this patient's condition?
- A. Streptococcus pneumoniae
 - B. Streptococcus Pyogenes
 - C. Moraxella catarrhalis
 - D. H. influenzae
- ANSWER: A
74. A 12-year-old schoolgirl was brought to a medical OPD with hearing impairment in both ears of two years duration. The impairment was progressive, and currently she could not hear any sound. Pure-tone audiometry showed profound hearing loss on both sides.
What is the next step in the management of this patient?
- A. Steroid trial
 - B. Hearing Aid
 - C. Sign language
 - D. Refer to ENT setting
- ANSWER: D
75. An eight-year-old boy was brought to a clinic by his parents with complaints of nasal discharge and headache. He also had an occasional dry cough of four months duration. He was given treatment a couple of times but with temporary relief of symptoms. Physical examination showed postnasal purulent discharge.
What is the most likely exacerbating factor for this patient's problem?
- A. Otitis media
 - B. Cystic fibrosis
 - C. Primary Ciliary Dyskinesia
 - D. Adenoidal hypertrophy
- ANSWER: D

76. A 28-year-old female patient presented to the clinic with foul-smelling nasal discharge of two years duration. She reported that her husband left her because of this problem. Examination showed the nasal cavity to be full of greenish-black dry crusts covering the turbinates and septum. Removal of the crust left a roomy and easily bleeding nasal cavity.

What is the next step in the management of this patient?

- A. Nasal saline douching
- B. Antibiotic nasal ointment
- C. Parenteral antibiotics
- D. Refer to ENT setting

ANSWER: D

77. A consultation was made for a 20-hour-old male neonate who had noisy breathing associated with blue spells. His condition improves when he cries. The duty physician could not pass the NG tube through both nostrils.

What is the most expected associated abnormality in this case?

- A. Hearing problem
- B. Polydactyl
- C. Dystopia canthorum
- D. Whiteforlock

ANSWER: A

78. A 60-year-old male patient presented to a hospital with a complaint of progressive swelling of the left side of his face of eight months duration. Physical examination showed a 6cm by 7cm, firm to hard left parotid mass that was ulcerated and fixed to the underlying surface.

For what condition is the patient at greatest risk after surgery?

- A. Bleeding
- B. Wound infection
- C. Keloid formation
- D. Facial palsy

ANSWER: D

79. An 18-month-old male child was brought to a clinic by his parents for a complaint of breathing difficulty associated with cough and fever of three days duration. He had a common cold attack six days ago. On physical examination, the child was anxious and restless with increased respiratory rate and biphasic stridor along with chest retraction.

What is the most likely etiology of the child's problem?

- A. Parainfluenza viruses
- B. Influenza A and B viruses
- C. Respiratory syncytial virus
- D. Adenovirus

ANSWER: A

80. A seven-year-old male child was brought to a clinic by his parents for a complaint of left ear pain of four hours duration. Otoscopic examination showed a small battery in the right external ear canal medial to the isthmus.

What is the most appropriate next step in the management of this patient's problem?

- A. Remove using an instrument
- B. Antibiotic drops
- C. Referral to ENT setting
- D. Remove using irrigation

ANSWER: C

81. A dentist performed a survey in a sample village and found multiple cases of tooth decay among the young and adolescents. The result of the survey also indicated the prevalence of tooth decay in that village to be 23 percent.

What is the most appropriate preventive strategy in this case?

- A. Topical fluoride application
- B. Use of fluoride mouthwashes
- C. Use of fluoride-based toothpaste
- D. Fluoridated community water supply

ANSWER: D

82. A 28-year-old female patient came to a dental OPD and complained of pain on her right upper back side of the jaw that wakes her at night and aggravates during mastication. Upon examination, she had a Class-II cavity on tooth number 25, which was tender on vertical percussion. Intraoral periapical x-ray shows widening of periodontal ligament space.

What is the most likely diagnosis of this patient?

- A. Reversible pulpitis
- B. Irreversible pulpitis
- C. Acute Apical Periodontitis
- D. Chronic Apical Periodontitis

ANSWER: C

83. A 24-year-old male patient came to an OPD with a complaint of swelling on the right back side of his face for one week, which showed an increment in size. On examination, the patient had swelling that extended from the lower border of the mandible to the infraorbital margin and from the anterior margin of the masseter muscle to the corner of the mouth. On aspiration, pus was drained from the swelling. Intraorally, tooth number 16 was grossly decayed, and it was tender on palpation and percussion.

What is the most likely diagnosis of this patient?

- A. Crohn's disease
- B. Buccal space infection
- C. Infraorbital space infection
- D. Retrozygomatic space infection

ANSWER: B

84. A 32-year-old male patient came to an emergency room with respiratory distress. On examination, he had swelling on the submandibular, submental, and sublingual areas bilaterally.

What is the most appropriate first-line antibiotic in the management of this patient?

- A. Cloxacillin 500 mg PO every 8 hours
- B. Gentamycin 80 mg IM every 12 hours
- C. Metronidazole 400 mg IV every 8 hours
- D. Penicillin G 2 to 4 million units IV every 6 hours

ANSWER: D

85. A 28-year-old male patient came to a dental OPD after he sustained a road traffic accident. On evaluation, he had laceration of the lower lip and forehead, and subluxation of tooth number 11.

What is the most appropriate next step in the management of the soft tissue injury in this case?

- A. His beard should be shaved
- B. His eyebrow should be shaved
- C. The dirt can be left within the wound
- D. Concentrated H₂O₂ can be used to clean the wound

ANSWER: A

86. A 68-year-old male patient came to a dental OPD with a complaint of a non-healing ulcer on his lateral border of the tongue for one month. The ulcer had irregular edges with indurations, pain, and bleeding, and it was fixated to the surrounding structures. There was also hypoglossal palsy and restriction of mobility of the tongue, progressive difficulty in mastication and speech, pooling of saliva, friability, and surface bleeding.

What is the most likely diagnosis of this patient?

- A. Sarcoma
- B. Lymphoma
- C. Malignant Melanoma
- D. Squamous cell carcinoma

ANSWER: D

87. A 25-year-old female patient visited a clinic for complaints of redness, tearing, and photophobia over the last four days. Examination of the left eye shows hyperemic conjunctiva and dendritic central corneal epithelial ulcer upon fluorescein staining. Corneal sensation was reduced in the affected eye.

What is the most appropriate management for this patient?

- A. Patch the eye with topical ointment
- B. Apply anesthetic and debride the cornea
- C. Give topical antibiotic and follow
- D. Refer to a nearby eye care center

ANSWER: D

88. A 38-year-old female patient was examined by a physician for progressive deterioration of her right eye vision over the last year. She gave a history of trauma with a stick. Examination shows dense whitish central corneal opacity.

What is the most appropriate next step in the management of this patient?

- A. Refer for investigation
- B. Refer for refraction
- C. Refer for follow-up
- D. Refer for corneal transplant

ANSWER: D

89. A 90-year-old woman came for follow-up at the ophthalmology clinic. On her chart, there was a visual field test attached that showed bilateral central visual scotoma with intact peripheral visual fields.

What is the most likely cause of the woman's condition in the above scenario?

- A. Glaucoma
- B. Optic atrophy
- C. Ocular tumors
- D. Macular degeneration

ANSWER: D

90. A 47-year-old hypertensive and diabetic patient came to an ophthalmic OPD after the sudden onset of painless visual reduction in the right eye of one day duration. Examination showed a right relative afferent pupillary defect with distance visual acuity of counting fingers at 3 meters. On retinal examination, there were tortuous retinal vessels with extensive dot and blot hemorrhages and macular edema in the right eye. Left eye findings were normal.
What is the most likely diagnosis of this patient?
- A. Diabetic retinopathy
 - B. Hypertensive retinopathy
 - C. Retinal vein occlusion
 - D. Retinal arterial occlusion
- ANSWER: C
91. A 23-year-old female patient visited an ophthalmology clinic for her annual eye checkup. She has had -4.00 diopter eyeglasses for the past few years.
What is the most likely diagnosis of this patient?
- A. Anisometropia
 - B. Myopia
 - C. Hyperopia
 - D. Astigmatism
- ANSWER: B
92. At a neonatal ICU, a two-day-old infant developed periorbital edema with hyperacute pronounced purulent eye discharge and hyperemic conjunctiva bilaterally.
What is the most likely diagnosis of this patient?
- A. Chemical conjunctivitis
 - B. Gonococcal conjunctivitis
 - C. Viral conjunctivitis
 - D. Chlamydial conjunctivitis
- ANSWER: B
93. A five-year-old girl was admitted to a pediatric ward for protein-energy malnutrition. On ocular examination, there was bilateral temporal bulbar conjunctival whitish growth with a dry corneal surface. Other ocular findings were normal.
What is the most likely diagnosis of this patient?
- A. Trachomatous scarring
 - B. Pingecula
 - C. Pterygium
 - D. Xerophthalmia
- ANSWER: D
94. A group of medical students were performing general health checkups for geriatrics at their nursing center. One of the students encountered a 77-year-old man with bilateral white pupils and an inability to walk alone because of his poor vision.
What is the most appropriate next step in the management of this man?
- A. Observe and re-evaluate
 - B. Send for further investigation
 - C. Arrange referral to eye center
 - D. Reassurance since it is an aging process
- ANSWER: C

95. A 35-year-old woman came to a clinic with complaints of itchy scalp and neck lesions, which started three days after applying hair dye. It is very itchy and has a burning sensation over it. On physical examination, there are erythematous oozing papules and plaques over the scalp, neck, and forehead with crusting and excoriation marks.

What is the most likely diagnosis of this patient?

- A. Irritant contact dermatitis
- B. Atopic dermatitis
- C. Psoriasis
- D. Allergic contact dermatitis

ANSWER: D

96. A 39-year-old lady presented with a facial skin lesion of six months duration. She reported that she has a burning sensation that is worsening during sun exposure. On examination, multiple erythematous papules and pustules over the central cheeks, forehead, and chin were observed.

What is the best management for this patient?

- A. Topical Metronidazole with Doxycycline
- B. Topical steroid with Doxycycline
- C. Topical retinoid with Doxycycline
- D. Topical retinoid with Azithromycin

ANSWER: A

97. A 28-year-old woman has experienced a skin lesion of eight months duration that is associated with mild itching. On physical examination, there are multiple well-demarcated, erythematous papules and plaques with silvery scale over the whole trunk and extensor side of the extremities, involving more than 30 percent of the body surface area. The Auspitz sign was positive.

What is the most effective treatment for this patient?

- A. Prednisolone
- B. Topical steroid
- C. Methotrexate
- D. Doxycycline

ANSWER: C

98. A 10-year-old girl is diagnosed with HIV after she presented with a complaint of itchy skin lesions of six months duration. On physical examination, she has hyperpigmented, discrete, and excoriated papules over the distal extremities symmetrically. There is also post-inflammatory hyperpigmented skin color change over the extremities. Her CD4 count was found to be 100 cells/mm³.

What are the most appropriate management steps for this patient?

- A. Start antiretroviral treatment, give topical steroid, and antihistamine
- B. Start antiretroviral treatment, prednisolone, and topical steroid
- C. Give topical steroid, antihistamine, and emollient
- D. Start Prednisolone, antihistamine, and 1 percent clotrimazole cream

ANSWER: A

99. A 12-year-old girl came to a dermatology outpatient department with a complaint of a skin rash that has been itchy for the last week. Further history reveals that she took an unspecified drug for abdominal pain 10 days ago. On physical examination, she has diffuse erythematous macules and small papules over the face and trunk.

What is the most effective management for this patient?

- A. Topical steroid and antihistamines
- B. Topical antibiotics and antihistamines
- C. Topical retinoids and antihistamines
- D. Topical vitamin D analog and steroid

ANSWER: A

100. A 34-year-old woman came to a dermatology outpatient department with a complaint of abnormal vaginal discharge of one week duration. She has associated dysuria and itching over the vagina. She is a commercial sex worker and had unprotected sex with a new customer two weeks back. On physical examination, there is minimal purulent discharge from the vagina, and on speculum there is purulent discharge from the cervix.

What are the most likely diagnosis and etiologic agent in this patient?

- A. Vaginal discharge syndrome – *Candida albicans* and *Trichomonas vaginalis*
- B. Vaginal discharge syndrome – *Neisseria gonorrhoeae* and *Chlamydia trachomatis*
- C. Vaginal discharge syndrome – *Treponema pallidum* and *Trichomonas vaginalis*
- D. Vaginal discharge syndrome – *Gardnerella vaginalis* and *Trichomonas vaginalis*

ANSWER: B

101. A 23-year-old female patient visited a dermatology clinic with a complaint of a skin lesion over her foot for two years. She has had pain for the last three months while walking. On physical examination, there are multiple hyperkeratotic papules over the soles of her right foot. There are also black dots on some of the lesions.

What is the most likely diagnosis of this patient?

- A. Molluscum contagiosum
- B. Plantar wart
- C. Flat wart
- D. Corn

ANSWER: B

102. A 13-year-old girl has experienced itchy skin lesion over the dorsum of her hand for two weeks. Initially it was a small red papule and later became larger to attain the current size. On examination, there is an annular plaque with scale and an active erythematous border with center clearing on the dorsum of her hand measuring 2cm by 4cm.

What is the most likely diagnosis of this patient?

- A. Tinea corporis
- B. Atopic dermatitis
- C. Nummular Eczema
- D. Lichen planus

ANSWER: A

103. A 23-year-old man was brought to an emergency room with an abnormal body movement. Upon evaluation, there was a generalized tonic movement of extremities associated with unresponsiveness. The family reported that they have seen the symptom for the last 20 minutes with no interruption in between.

What is the most likely diagnosis of this case?

- A. Non-convulsive seizure
- B. Status epilepticus
- C. Epilepsia partialis continua
- D. Temporal lobe seizures

ANSWER: B

104. A 26-year-old man was involved in a road traffic accident and sustained severe soft tissue injury to both legs. He was put on oxygen and one bag of crystalloid resuscitation at a medical center. After 30 minutes, he was referred to a tertiary setup for better care. On presentation at the hospital, he is in pain and has a blood pressure of 80/50 mmHg which has not markedly improved.

What is the most appropriate next step in the management of this patient?

- A. Dopamine infusion
- B. Noradrenaline infusion
- C. Blood transfusion
- D. Continue crystalloid infusion

ANSWER: C

105. A 45-year-old male patient with chronic liver disease was presented to the ER with a one week history of inability to ambulate and nonsensical conversation. He is on follow-up at the GI clinic, and his family reported that he is adherent to his medications. He has no vomiting or diarrhea. On evaluation, his blood pressure is 90/60 mmHg, pulse rate is 110/min, respiratory rate is 14/min, and temperature is 36.5 degrees Celsius. A physician suspected hepatic encephalopathy and planned to admit him to the ER.

What is the most likely cause for the patient's deterioration?

- A. Carbohydrate load from a large meal
- B. Occult GI bleeding
- C. Spontaneous bacterial peritonitis
- D. Hypokalemia

ANSWER: B

106. A 50-year-old man came to a clinic for a general checkup. On evaluation, his blood pressure is 210/100 mmHg, pulse rate is 101/min, respiratory rate is 32/min, oxygen saturation is 93 percent, heart sounds are normal, and his chest is clear.

What is the most likely diagnosis of this patient?

- A. Massive pulmonary embolism
- B. Hypertensive urgency
- C. Hypertensive emergency
- D. Aortic dissection

ANSWER: B

107. A 73-year-old female known Type-II DM patient was brought to an emergency OPD with severe shortness of breath and epigastric discomfort of one day duration. On examination, she is in cardiorespiratory distress with blood pressure of 90/60 mmHg, a pulse rate of 123/min, a respiratory rate of 34/min, an oxygen saturation of 78 percent, a temperature of 38.2 degrees Celsius, and her chest is full of rales bilaterally. On investigation, BNP was 200 pg/ml.

What is the most likely explanation for this condition?

- A. Non-cardiogenic pulmonary edema
- B. Acute respiratory distress syndrome
- C. Cardiogenic pulmonary edema
- D. Pulmonary embolus

ANSWER: C

108. A 58-year-old diabetic patient with a previous history of acute coronary syndrome was rushed to the ER with a four hours history of shortness of breath and diaphoresis. On examination, he is acutely sick-looking with a blood pressure of 150/95mmHg, a pulse rate of 101/min, a respiratory rate of 34/min, a temperature of 37 degrees Celsius, and an oxygen saturation of 85 percent. The nurses put him on supplemental oxygen and performed an urgent ECG, for which the findings indicated tombstone ST-elevation of V1-V6, Lead I, and AVL.

What is the most likely explanation for the ECG finding?

- A. Anterolateral STEMI
- B. Inferolateral STEMI
- C. Posterior wall STEMI
- D. Right ventricular STEMI

ANSWER: A

109. A 34-year-old man, who had been to a lowland area a week back, was referred from a health center to a hospital with a history of fever and confusion of two days duration. Before his presentation, he was apparently healthy. While being examined in the ER, he had a brief convulsion and remained obtunded afterwards. Laboratory results indicated that he has a WBC count of $15,000/\text{mm}^3$ and a hemoglobin level of 10.1 gm/dl.

What is the most likely explanation for this case?

- A. Cryptococcal meningitis
- B. Cerebral malaria
- C. TB meningitis
- D. Pyogenic meningitis

ANSWER: B

110. A 28-year-old primigravid mother at the 30th week of gestational age experienced blurring of vision of one day duration with associated convulsions of four episodes. She had no history of any chronic illness before this complaint. Head CT showed hypodense lesions at the gray-white matter junction on the parieto-occipital lobe.

What is the most likely diagnosis of this patient?

- A. Stroke
- B. Pre-eclampsia
- C. Eclampsia
- D. HELLP syndrome

ANSWER: C

111. A 25-year-old primigravida at the 37th week of gestational age was brought to an emergency room after she had four episodes of active convulsions. She was loaded with magnesium at the emergency.

What is the expected time of regaining consciousness after magnesium loading?

- A. 1-2
- B. 3-4
- C. 5-6
- D. 7-8

ANSWER: A

112. A 28-year-old primigravida at the 28th week of gestational age presented with altered mental status and three episodes of abnormal body movement. The patient was then immediately loaded with magnesium.

Why is magnesium the drug of choice in this case?

- A. Wide margin of safety for the fetus
- B. High success rate in controlling the seizure
- C. Prevention of recurrent seizure
- D. Strong anti-hypertensive effect

ANSWER: B

113. A 20-year-old passenger sustained a road traffic accident and was immediately brought to an emergency room. On examination, a respiratory rate of 45/min with room air and a paradoxical pattern of breathing were noted.

What is the most likely diagnosis of this patient?

- A. Flail chest
- B. Pneumothorax
- C. Hemothorax
- D. Blunt cardiac injury

ANSWER: A

114. A 30-year-old male patient, who lives in a conflict area, sustained a bullet injury to his abdomen. He was immediately taken to a primary hospital for urgent evaluation and management.

What is the most likely affected organ of this patient?

- A. Liver
- B. Spleen
- C. Small Intestine
- D. Large Intestine

ANSWER: C

115. A 40-year-old man, who became unconscious on the street, was brought to an emergency room by a policeman. Examination findings revealed a mean arterial pressure of 80, a pulse rate of 90/min, and an SPO₂ of 92 percent with atmospheric air. Besides, a 3cm by 2cm fresh wound around the umbilical area with visible small eviscerated omentum, mid-sized and reactive pupils and a GCS level of 8 were noted.

What is the most likely indication for taking this patient immediately to the operating room?

- A. Hemodynamic status
- B. The eviscerated omentum
- C. The patient's consciousness
- D. The local wound exploration

ANSWER: B

116. A 55-year-old patient was hit by a vehicle while crossing the road. He was then immediately brought to an emergency room. After resuscitation, a urinary Foley catheter was inserted, and frank blood was coming out of the catheter. Pelvic x-ray shows Tile C pelvic fracture.

What is the most appropriate next step in the management of this patient?

- A. Exploration and repair
- B. Conservative management
- C. Suprapubic cystotomy
- D. Antibiotics

ANSWER: B

117. A 25-year-old man sustained a head injury while he was working in a cement factory. On evaluation, his blood pressure was 170/90 mmHg, pulse rate was 58/min, and GCS was E-2, V-3, and M-4. He moves only his left upper and lower extremities when painful stimuli are applied, and he has a 2 cm by 3 cm depressed skull fracture on the right side. Head CT showed a 2 cm depressed skull fracture with minimal epidural collection at the fracture site.

What is the most likely indication for surgery in this patient?

- A. Depressed skull fracture
- B. Bossy weakness
- C. Acute epidural hematoma
- D. Increased blood pressure

ANSWER: B

118. A 28-year-old man sustained a head injury while he was at work on construction. He was then brought to an emergency department for evaluation and management. On evaluation at the emergency department, he has blood pressure of 150/90 mmHg, projectile vomiting, and a headache followed by immediate deterioration of GCS.

What is the most appropriate next step of management for the above condition?

- A. Fluid management
- B. Blood transfusion
- C. Mannitol 1 gm/kg loading
- D. Tramadol 100mg IV

ANSWER: C

119. A 46-year-old woman sustained a road traffic accident and was immediately taken to an emergency department for evaluation and management. On neurologic examination, there was motor weakness in the upper and lower extremities. The weakness was more pronounced on the upper extremities with associated decrease in pain and temperature sensation.

What is the most likely diagnosis of this patient?

- A. Brown-Sequard syndrome
- B. Central cord syndrome
- C. Anterior cord syndrome
- D. Cauda Equina syndrome

ANSWER: B

120. A 35-year-old male driver came to an emergency room with red, painful, and tender facial skin secondary to prolonged exposure to sunlight.
What is the most likely degree of burn in this patient?
- A. First degree
 - B. Superficial second degree
 - C. Deep second degree
 - D. Third degree
- ANSWER: A
121. A 70kg man sustained a thermal burn while he was trying to extinguish a fire that started in one of the nearby houses. He was immediately taken to a primary hospital by his neighbors. Upon examination, he has burns over the entire posterior trunk and entire left leg. Fluid management was started.
What is the most appropriate amount of fluid to be given in the first 24 hours?
- A. 5,040 ml
 - B. 7,320 ml
 - C. 10,080 ml
 - D. 15,120 ml
- ANSWER: C
122. A two-year-old male child experienced a sudden onset of cough and shortness of breath for two hours; then, he was then taken to an emergency OPD. Upon evaluation, he had a wheeze on his left chest. Chest x-ray showed hyperinflation of the left chest.
What is the most likely cause of this case?
- A. Pneumonia
 - B. Heart failure
 - C. Asthma
 - D. Foreign body aspiration
- ANSWER: D
123. An eight-year-old boy accidentally took isoniazid which his father was taking. The boy was then immediately brought to an emergency room for evaluation and management.
What antidote should be given to the boy?
- A. Naloxone
 - B. Atropine
 - C. Calcium gluconate
 - D. Pyridoxine
- ANSWER: D
124. A nine-year-old boy was hit on his chest by a car while he was crossing a road. He was then immediately taken to the emergency department. On evaluation, he has tachycardia, hypotension, distended neck veins, and muffled heart sounds. He was started on fluids but responded poorly.
What is the most likely type of shock in this case?
- A. Septic shock
 - B. Obstructive shock
 - C. Anaphylactic shock
 - D. Neurogenic shock
- ANSWER: B

125. A five-day-old neonate was brought by his mother to an emergency room after he manifested fever, tachypnea, and shortness of breath. After evaluation, the neonate was diagnosed with pneumonia.

What is the most likely etiology of pneumonia in this neonate?

- A. *Streptococcus pneumoniae*
- B. *Chlamydia trachomatis*
- C. *Mycoplasma pneumoniae*
- D. *Listeria monocytogenes*

ANSWER: D

126. A seven-year-old child, who has a known cardiac disease and has been on follow-up for the past three years, sustained cardiac arrest in the emergency department. CPR was immediately started.

What should be the compression-ventilation ratio in this case?

- A. 30:2
- B. 15:2
- C. 30:1
- D. 15:1

ANSWER: B

127. An infection prevention team of a hospital developed a healthcare waste strategy for the purpose of infection prevention through effective waste management. Consequently, the team distributed responsibilities for the different bodies in the hospital for the various steps of healthcare waste management.

Who are the most appropriate persons responsible for segregating the waste?

- A. Trained cleaners
- B. Waste producers
- C. Healthcare workers
- D. Infection prevention team

ANSWER: B

128. The number of people affected with malaria infection has increased in an area despite the efforts to reduce the burden associated with the disease. Public health officials of the area requested service providers to solicit reports of the disease by contacting the patients and taking blood slides for further prevention and control.

What is the most likely surveillance system implemented in the above scenario?

- A. Active surveillance
- B. Passive surveillance
- C. Sentinel surveillance
- D. Serological surveillance

ANSWER: A

129. A district health manager received a report indicating the onset of an outbreak of diarrheal disease. Consequently, the manager established a team to confirm whether the occurrence is a real epidemic or not.

What is the most appropriate first step the team should perform in this scenario?

- A. Confirming if the diagnosis is correct
- B. Confirming the type of outbreak occurring in the area
- C. Confirming if the increase in the number of cases is real
- D. Confirming if the increase in the cases represents an outbreak

ANSWER: A

130. In a metropolitan city, a significant proportion of the adolescent population is being diagnosed with HIV/AIDS and other sexually transmitted infections. This is primarily attributed to increased urbanization and migration of people from rural to urban areas.

What is the most likely factor that increased the risk of infection in this case?

- A. Socio-demographic
- B. Biologic
- C. Behavioral
- D. Environmental

ANSWER: A

131. A health educator was requested to prepare health education materials for high school students in the form of a story so that the students will better understand the messages.

What is the most important characteristic of a good story that should be considered by the educator?

- A. People in the story should have names
- B. The story should be long enough to convey the message
- C. The actions in the story should be strange to attract the audience
- D. The story should include names of real people to make it believable

ANSWER: A

132. In a health center, a newly diagnosed TB patient was prescribed a medication that is going to be taken for a long period of time. Even though the patient needs to remember to take the medicine each day, the patient feels confident and competent in his ability to overcome perceived barriers of the prescribed treatment.

What is the most likely factor that influences the patient's behavior?

- A. Self-efficacy
- B. Outcome-efficacy
- C. Behavioral intention
- D. Outcome expectation

ANSWER: A

133. A government body in an area took corrective measures on social and cultural patterns, including nightclubs, which contribute to an elevated risk of communicable diseases. The corrective measures were taken on the basis of the recommendation proposed by researchers.

What is the most likely level of prevention applied in the above scenario?

- A. Primary
- B. Secondary
- C. Tertiary
- D. Primordial

ANSWER: D

134. An environmental health officer was requested to design a wastewater treatment plant for a newly constructed small-sized staff cafeteria. The officer decided to have an anaerobic treatment system for the organic wastes generated from the cafeteria so that the effluent will later be subjected to a polishing treatment.

What is the most likely wastewater treatment type considered by the officer?

- A. Septic tank system
- B. Seepage pit system
- C. Activated sludge system
- D. Waste stabilization pond

ANSWER: A

135. A physician was investigating the cause of an outbreak that occurred in a sample district. After conducting the investigation, the physician, together with other team members, reached a conclusion that the source of the outbreak is a contaminated nearby water well. What is the most appropriate first measure to control the outbreak?
- A. Treating infected cases
 - B. Treating the water source
 - C. Educating the community
 - D. Closure of the water source
- ANSWER: D
136. An unidentified outbreak with a significant increment of cases every day was reported from a health extension worker to the district health office. The epidemic investigation team confirmed that the outbreak is malaria, which is transmitted from person to person by mosquito vectors. What is the most likely type of outbreak that happened in the area?
- A. Mixed
 - B. Progressive
 - C. Continuous
 - D. Point source
- ANSWER: B
137. A local NGO designed a new stove for rural women in order to reduce indoor air pollution. The NGO distributed the first round of the products to households in one *Kebele* so that the community would adopt the new stove. The organization planned to reach out with the products to all community members in the *Woreda*. What is the most appropriate theory applied in the above scenario?
- A. Social cognitive
 - B. Reasoned action
 - C. Trans-theoretical
 - D. Diffusion of innovations
- ANSWER: D
138. An environmental health officer collected water samples from a community water supply for chemical analysis. The laboratory investigation showed that the water contains a chemical that has a benefit and harm to the teeth and bone at lower and higher concentrations, respectively. What is the most likely chemical found in the water?
- A. Iron
 - B. Arsenic
 - C. Fluoride
 - D. Manganese
- ANSWER: C
139. A *Woreda* health office plans to assess the prevalence of COVID-19 and its associated factors among residents of a sample *Kebele*. The list of all residents in the *Kebele* is known, and the sample size has been determined to select the study participants. What is the most appropriate sampling technique to be used for this study?
- A. Simple random sampling
 - B. Systematic random sampling
 - C. Stratified random sampling
 - D. Multistage random sampling
- ANSWER: A

140. A zonal health office decided to share financial resources donated by a non-governmental organization to support HIV/AIDS programs. Hence, the office planned to undertake research in order to estimate the burden of HIV/AIDS in each *Woreda*.

What is the most appropriate study design to be used for this research?

- A. Cross-sectional
- B. Case series
- C. Case report
- D. Ecological

ANSWER: A

141. Malaria cases in a rural community are increasing from year to year, and people are dying due to malaria. A researcher assumed that this may be due to poor bed net utilization and planned to assess bed net utilization in this rural community.

What is the most appropriate data collection tool for this study?

- A. Observation
- B. Questionnaire
- C. Interviewee guide
- D. In-depth interview

ANSWER: A

142. A male medical student is doing his research for fulfillment of his medical doctorate degree. His research topic is entitled “Assessing Sexual Abuse and Predisposing Factors among College Students.” He is asking for a suggestion on how to secure ethical issues during his assessment.

What is the most important step to secure ethical principles during the study?

- A. Code the questionnaires by name
- B. Avoid disseminating the finding
- C. Include study participants by coercion
- D. Respect for the study participants’ autonomy

ANSWER: D

143. A non-governmental organization planned to support children in a rural community through a nutritional program. As part of the program, the organization needs to generate baseline data related to malnutrition.

What is the most appropriate research objective for this scenario?

- A. To identify factors associated with malnutrition
- B. To assess the trend of malnutrition in the area
- C. To estimate the cost of intervention in the area
- D. To determine the magnitude of malnutrition

ANSWER: D

144. A researcher planned to test the claim that the mean BMI of patients with Type-I and Type-II diabetes is equal in a referral hospital. Random samples of 25 Type-I and 36 Type-II diabetes patients were selected. The mean BMIs of the two groups were 20 kg/m² and 25 kg/m², with standard deviations of 4 and 6 for Type-I and Type-II diabetes patients, respectively.

What is the most appropriate test statistic used to test the given hypothesis?

- A. Z statistic
- B. t statistic
- C. F statistic
- D. Chi-square

ANSWER: B

145. A researcher wishes to study whether there is an association between drinking coffee and atherosclerosis. He applied a case-control study design and calculated an odds ratio of 0.8. What is the most likely association between drinking coffee and atherosclerosis in this study?
- A. No association
 - B. Protective by 20 percent
 - C. Risk increases by 20 percent
 - D. Protective by 0.8 times
- ANSWER: B
146. A district public health emergency officer is asked by the unit coordinator to summarize and present the last five years pattern of malaria prevalence rate of the district. What is the most appropriate diagram to be used in this case?
- A. Pie chart
 - B. Bar chart
 - C. Line graph
 - D. Histogram
- ANSWER: C
147. A 30-year-old pregnant woman at 34-week gestational age came to labor and delivery unit with pushing-down pain. Cesarean section was indicated to save the baby's life. Despite understanding that her baby may die unless a cesarean section is performed, she refused the treatment. What is the most appropriate action in this case?
- A. Consult her partner and perform the Cesarean section
 - B. Perform the Cesarean section to save the baby's life
 - C. Inform the hospital CEO and perform the Cesarean section
 - D. Consider further counselling the woman to obtain her consent
- ANSWER: D
148. A 40-year-old woman who is paralyzed and entirely dependent on respiratory support refused to continue medical treatment. She had been repeatedly advised by the physician that she would die if she discontinued the treatment. What is the most appropriate action the physician should take in this scenario?
- A. Discuss with the hospital management to arrive at a meaningful decision
 - B. Provide the appropriate treatment irrespective of her consent
 - C. Withhold the respiratory support as per her preference
 - D. Refer to other health institution
- ANSWER: C
149. A 51-year-old male patient is admitted to a surgical ward due to burn injury on his right leg. The physician ordered daily wound care to facilitate wound healing. On the following day, while the physician was doing patient rounds, he noticed that the wound care was done for the patient that morning. However, the physician wanted to get information regarding the wound healing process. What is the most important activity the physician should do in this case?
- A. Ask the assigned nurse for the progress of the wound healing
 - B. Remove the dress and observe the wound and redress the wound
 - C. Ask the patient about the progress of his wound
 - D. Plan to come earlier the next day and observe the wound during wound care
- ANSWER: A

150. A 27-year-old male patient came to an outpatient department with abdominal pain and distention. Following investigation findings, a physician diagnosed intestinal obstruction and planned surgical treatment.

What is the most important point the physician should consider to avoid litigation?

- A. Provide detailed information on the importance of the surgical treatment
- B. Reassure the patient for a better surgical outcome
- C. Let the patient sign the consent form
- D. Counsel on the alternative treatments

ANSWER: D

151. A 65-year-old male patient, who was admitted to an inpatient ward, was advised by a doctor to stay in bed and take a rest. However, when the doctor came back, the patient was not on his bed. The doctor got angry and started a conflicting argument when the patient returned.

What is the most likely step of communication the doctor should have applied in this case?

- A. Clarify the conflict
- B. Use objective criteria where possible
- C. Brainstorm about possible solution
- D. Focus on common interest, not position

ANSWER: A

152. A mother of an eight-month-old infant went to a minor surgical OPD for circumcision. Following the circumcision, the infant was bleeding, febrile, jaundiced, and anemic with continuous bleeding. A senior physician was consulted, and he ordered a full blood count to see if there is any underlying problem. The infant turned out to be hemophilic, and proper treatment was initiated.

What is the most appropriate action to be done to avoid similar incidents in the future?

- A. Circumcision procedures should be performed only by seniors
- B. The procedure is risky, so patients have to wait till they are getting old to know their medical conditions
- C. Rare diseases are not likely to happen in everyday cases. Hence, it should not be a problem
- D. Thorough laboratory investigation and family history must be taken before any operative surgery

ANSWER: D

153. A male physician working at a hospital with COVID-19 patients is confused about the constant changing of symptoms of the illness among different patients. But he was convinced that he needed to be at peace since the illness was new and lots of facts were yet to be known.

What is the most likely principle of compassionate care demonstrated by the physician in this case?

- A. Attention
- B. Acknowledgement
- C. Acceptance
- D. Affection

ANSWER: C

154. A medical director of a hospital observed that patient demand and need are progressively increasing. The director also observed that some of the hospital staff members started to come late and their motivation for work is declining. Patients also complain that they are not getting the proper care.

What is the most effective way to handle this situation?

- A. Dealing with the staff stress and burnout
- B. Dealing with the wider health facility context
- C. Teaching compassion to the staff through training
- D. Developing an inner compassionate self

ANSWER: A

155. A 45-year-old male patient came to OPD with complaints of productive cough with whitish sputum, low-grade fever, significant weight loss, and night sweats for three weeks. Sputum smear, sputum GeneXpert, sputum culture, and pleural fluid GeneXpert were ordered.

What is the most sensitive test for this patient's diagnosis?

- A. Sputum culture
- B. Sputum smear microscopy
- C. Pleural fluid GeneXpert
- D. Sputum GeneXpert

ANSWER: A

156. A 56-year-old male patient from prison came to a medical OPD with a complaint of difficulty of walking for two weeks. He also had a history of anorexia, weight loss, and fatigue, which started a month back, and lower back pain for three months. Examination revealed tenderness over the upper lumbar area, and lower extremity power is 1/5 bilaterally, DTR is $\frac{3}{4}$, and there is a positive Babinski sign. Thoracolumbar CT shows deformed morphology with anterior wedging at L1 and L2 vertebrae impinging the cord, features of epiphysitis at upper- and lower-disc regions, no sclerosis, and no punched-out lesions.

What is the most likely diagnosis of this patient?

- A. Multiple myeloma
- B. Tuberculous spondylitis
- C. Brucellosis
- D. Prostatic cancer

ANSWER: B

157. A 35-year-old female known RVI patient who discontinued her HAART for four months back presented with progressive dyspnea and a dry cough of two weeks duration. Her current CD4 count is 185 cells/mm³. Her physical examination reveals fever, tachypnea, SpO₂ of 85 percent, and scattered crepitation in the lungs bilaterally. Chest x-ray shows perihilar interstitial infiltration, which spreads outward.

What is the most likely explanation for this patient's presentation?

- A. Tuberculosis
- B. Interstitial lung disease
- C. Pneumocystis pneumonia
- D. Lymphoid interstitial pneumonitis

ANSWER: C

158. A 40-year-old male patient came to an emergency room with an intermittent fever of three days duration. He has associated chills and rigor. He has travel history to a lowland area two weeks back. He was kept at an emergency room, and his fever was spiking every 72 hours.

What is the most likely diagnosis of this patient?

- A. Typhoid fever
- B. Relapsing fever
- C. Typhus
- D. Malaria

ANSWER: D

159. A 30-year-old male patient visited a health center with prostration, severe headache, fever, and prominent nonproductive cough of three days duration. His physical exam shows a generalized rash involving the entire body except the face, palms, and soles. Skin necrosis and gangrene of the digits are also noted on physical examination.

What is the most appropriate preventive mechanism for the diagnosis of this patient?

- A. Control of body lice
- B. Using bed nets
- C. Vaccination
- D. Avoiding contaminated food

ANSWER: A

160. A 30-year-old male patient came to a hospital with complaints of dyspnea and cough of one month duration. He has associated palpitation and bilateral leg swelling. His physical exam shows raised JVP, hyperdynamic precordium, and murmur of mitral stenosis.

What is the most likely cause of this patient's problem?

- A. Dilated cardiomyopathy
- B. Rheumatic heart disease
- C. Hypertrophic cardiomyopathy
- D. Ischemic cardiomyopathy

ANSWER: B

161. A 20-year-old male patient, who had previous migratory joint pain, presented with abnormal dance-like movements of extremities of three weeks duration. He has associated dyspnea and palpitation. His physical exam shows bibasilar rales and Grade-I pitting bilateral edema.

What is the most appropriate initial management for this patient?

- A. Aspirin
- B. Diuretics plus Carbamazepine
- C. Diuretics plus Aspirin
- D. Diuretics plus Prednisolone

ANSWER: B

162. A 30-year-old male patient known to have rheumatic heart disease (MS, MR, and AS) complained of fever and fatigue of three weeks duration. He has no previous history of cardiac surgery. His echocardiography study shows oscillating mass at the mitral valve, and the blood culture result is pending. What is the most appropriate empirical treatment for this patient?

- A. Vancomycin plus Gentamicin
- B. Vancomycin plus Ceftriaxone
- C. Vancomycin plus Gentamicin plus Cefepime
- D. Ceftriaxone

ANSWER: B

163. A 50-year-old male patient, who regularly chews khat, complained of left-side chest pain of one day duration. He used to drink alcohol and smoke cigarettes. His coronary angiography, which is done after two days of his presentation, shows luminal narrowing of the left anterior descending coronary artery by 80 percent. His laboratory test shows high HDL and low LDL levels. What is the most likely factor that has the highest relative risk for the development of this patient's problem?
- A. Alcohol drinking
 - B. Cigarette smoking
 - C. Chat chewing
 - D. High HDL
- ANSWER: B
164. A 45-year-old female patient presented to OPD with a two months history of bilateral wrist joint pain and morning-time hand stiffness that lasts for one hour. On workup, the wrist x-ray is normal, the rheumatoid factor is negative, and the ESR is 80 mm/hr. The duty physician decided to do further laboratory investigation to confirm the diagnosis. What is the next appropriate laboratory test in this case?
- A. CRP
 - B. ANA
 - C. Anti-CCP antibodies
 - D. Anti-double strand DNA
- ANSWER: C
165. A 45-year-old male patient presented with intermittent diarrhea of one month duration. Three months back, he had surgery for extensive gangrenous small bowel obstruction involving mainly the ileum. Which micronutrient deficiency would the patient most likely develop?
- A. Iron
 - B. Folic acid
 - C. Calcium
 - D. Cobalamin
- ANSWER: D
166. A 35-year-old man came with complaints of epigastric pain, which is of a burning character. He explains that the pain occurs two hours after a meal and is frequently relieved by antacids or food. The pain also awakens him from sleep at midnight. However, the patient is well and has no comorbidity before. On examination, he has epigastric tenderness. What is the preferred approach in the management of this patient?
- A. Do endoscopy
 - B. Have a barium study
 - C. Test for *H. pylori* and treat
 - D. Treat with PPI
- ANSWER: C

167. A 40-year-old man visited a hospital with a complaint of abdominal swelling of two months duration. He has a history of jaundice recently. On laboratory, liver enzymes and bilirubin levels are elevated, hemoglobin level is 10 g/dl and platelet count is 100,000/microliter.

What laboratory finding most likely suggests chronic liver disease in this patient?

- A. Elevated bilirubin
- B. Elevated enzymes
- C. Hemoglobin level
- D. Platelet count

ANSWER: D

168. A 20-year-old male patient came to a medical OPD with a complaint of body swelling of one week duration. The swelling started from the face and involved the extremity as well. He also complains of reddish discoloration of urine and reduction in urine amount. He denied any previous illnesses. His blood pressure is 140/90 mmHg, urinalysis shows blood +3 and protein +2 with RBC casts.

What is the most likely underlying cause of the patient's condition?

- A. Endocarditis-associated glomerulonephritis
- B. Poststreptococcal glomerulonephritis
- C. IGA nephropathy
- D. Lupus nephritis

ANSWER: B

169. A 35-year-old man came to a medical OPD with complaints of left flank pain of two days duration, which is associated with urinary frequency. He also has loss of appetite and fever, but no vomiting. He has no history of previous similar complaints. On laboratory, a WBC count of 12,000/microliter with a left shift and many WBCs on urinalysis were seen.

What is the most appropriate diagnostic workup to be strongly recommended for this patient?

- A. Urine culture
- B. Blood culture
- C. Abdominal U/S
- D. Further workup not needed

ANSWER: C

170. A 30-year-old woman visited a health center with complaints of fatigue and weight loss despite a good appetite for one month. She has associated hot intolerance, sweating, palpitation, and oligomenorrhea. She has no fever or headache. On examination, there is a small goiter on her neck.

What is the most appropriate workup to reach the diagnosis?

- A. Neck ultrasound
- B. FNAC
- C. Serum TSH
- D. Radionuclide scan

ANSWER: C

171. A 55-year-old man complained of polyuria and polydipsia of one month duration. He was diagnosed with pneumonia a week before. Currently, the symptoms worsened, and he developed confusion. On examination, he has a dry tongue and buccal mucosa, his pulse rate is 112/min, and his GCS is 14/15. Laboratory tests show RBS of 600 mg/dl, and glucose +3, and trace ketone on urinalysis. Other lab test results are pending.

What is the most likely clinical diagnosis of this patient?

- A. Type 1 DM with DKA
- B. Type 1 DM with acute hyperglycemia
- C. Type 2 DM with DKA
- D. Type 2 DM with HHS

ANSWER: D

172. A physician managing a diabetic patient is planning to achieve strict glycemic control in collaboration with the patient.

What is the most likely complication of diabetes reduced with this method?

- A. Coronary heart disease
- B. Diabetic nephropathy
- C. Peripheral arterial disease
- D. Cerebrovascular disease

ANSWER: B

173. A 45-year-old female patient complained of easy fatigability of one month duration. On further inquiry, he reported that he had intermittent diarrhea and crampy abdominal pain for the past four months. Examination revealed a blood pressure of 100/60 mmHg, a pulse rate of 98/min, a temperature of 36.5 degrees Celsius, pale conjunctivae, and a clear chest, but there is no organomegaly or abdominal tenderness. Laboratory evaluation revealed a hemoglobin level of 9.0 mg/dl with MCV of 74 fl, creatinine of 1.0 mg/dl, viral markers were negative, no o/p on stool exam, and stool occult was negative. Abdominal ultrasound showed ileal thickening with no lymphadenopathy.

What is the most likely cause of anemia in this patient?

- A. Heavy menses
- B. Acute blood loss
- C. Malabsorption
- D. Renal disease

ANSWER: C

174. A 50-year-old male patient presented with easy fatigability, loss of appetite, high-grade fever, and significant weight loss of two months duration. Examination revealed a blood pressure of 100/60 mmHg, a pulse rate of 109/min, a temperature of 37.9 degrees Celsius, pale conjunctivae, and a clear chest, but no organomegaly or abdominal tenderness. Laboratory findings revealed a WBC count of $1000/\text{mm}^3$ with 50 percent neutrophils, a hemoglobin level of 8.5 mg/dl, a platelet count of $35,000/\text{mm}^3$, and a creatinine level of 1.1 mg/dl. Peripheral morphology showed suspicious blast cells. Liver panel tests and urinalysis findings are non-revealing.

What is the most appropriate next diagnostic step in the evaluation of this patient?

- A. Bone marrow examination
- B. Abdominal ultrasound
- C. Ph chromosome
- D. LP

ANSWER: A

175. A 25-year-old girl was presented to an emergency room after experiencing abnormal body movement of three minutes duration. It is characterized by involving the left upper extremities initially and progressed to affect all extremity with flexion-extension movement, and the patient had impaired consciousness during the episode.
What is the most likely clinical diagnosis of this patient?
- A. Primary generalized tonic-clonic seizure
 - B. Status epilepticus
 - C. Evolved generalized seizure from focal
 - D. Focal seizure with impaired awareness
- ANSWER: C
176. A 40-year-old male patient who has a previous history of infective endocarditis came to a medical OPD for advice concerning prophylactic antibiotics before surgical procedures.
Which surgical procedure needs antibiotic prophylaxis in this case?
- A. Gastrointestinal surgery
 - B. Genital tract surgery
 - C. Dental surgery
 - D. Ophthalmic surgery
- ANSWER: C
177. A 23-year-old female patient, who complains of a week-long weakness of her extremities, came to a medical OPD. The weakness is associated with a tingling sensation that started from her lower leg and later involved all of her limbs. Neurologic examination reveals quadriplegia (power of 1/5 in lower extremities and 2/5 in upper extremities); biceps and ankle reflexes are absent. Temperature and pain sensations are intact.
What is the most likely diagnosis of this patient?
- A. Dermatomyositis
 - B. Poliomyelitis
 - C. Myasthenia gravis
 - D. Guillain-Barre syndrome
- ANSWER: D
178. A 32-year-old male patient came to a hospital with complaints of pleuritic chest pain and dyspnea of two months duration. Chest examination findings show dullness on percussion and absent air entry basally.
What is the most appropriate investigation modality for this patient?
- A. Thoracentesis
 - B. Lateral decubitus chest X-ray
 - C. Chest ultrasound
 - D. Chest CT scan
- ANSWER: C

179. A 24-year-old man presented with fever, pleuritic chest pain, and dyspnea of three weeks duration. Chest examination findings show dullness on percussion and absent air entry. Investigation findings show a white blood cell count of $12,000/\text{mm}^3$, a hemoglobin level of 10.7 mg/dl, an ESR of 55 mm/hr, and a chest X-ray revealed effusion. Thoracentesis shows straw-colored fluid, a WBC count of $600/\text{mm}^3$ with 67 percent lymphocytes, and protein of 4 g/dl.

What is the best way to establish the diagnosis of this patient?

- A. Needle pleural biopsy
- B. Pleural fluid culture
- C. Pleural fluid ADA level
- D. Pleural fluid cytology

ANSWER: A

180. A 27-year-old man, who had tuberculosis treatment four years ago, complained of hemoptysis and a cough productive of thick sputum of three weeks duration. His investigation is notable for an ESR of 45 mm/hr, and hemoglobin level of 7.5 mg/dl. Chest x-ray shows bronchiectasis.

What is the most appropriate initial step in this patient's care?

- A. Send for a chest CT scan
- B. Send for sputum Gram stain and culture
- C. Empirically start on antibiotics
- D. Consider surgical intervention

ANSWER: B

181. A chief executive director of a health science college drew his attention to the structure and process of allocating jobs in order to achieve common objectives. After arranging different teams and materials to get things done on time, the director delegated the deputy director to follow all the activities.

What is the most likely management function applied by the director?

- A. Staffing
- B. Directing
- C. Organizing
- D. Controlling

ANSWER: C

182. A medical director, who is working in a tertiary hospital, tries to balance work distribution among health professionals in order to make healthcare services more effective.

What is the most likely principle of management used in this case?

- A. Division of labor
- B. Convergence of work
- C. Management by exception
- D. Shortest decision path

ANSWER: A

183. A planning officer from the Ministry of Health was invited to give training to regional health bureau officers. The focus of the training was on the preparation of a strategic plan. The officer explained about goals, aims, objectives, and targets that can be incorporated in a strategic plan and gave the following example at the end of the training: "By the end of 2024, 95 percent of eligible children will be vaccinated against six target diseases."

What is the most likely element of the strategic plan denoted in the above example?

- A. Aim
- B. Goal
- C. Target
- D. Objective

ANSWER: C

184. A primary hospital planned to hire health professionals for vacant positions. The human resource department announced the jobs through newspapers, and many health professionals applied for the positions.

What is the most appropriate human resource management activity applied by the department?

- A. Selection
- B. Recruitment
- C. Socialization
- D. Job Induction

ANSWER: B

185. A female medical director of a specialized hospital is well recognized for her leadership skills. She is assertive and decisive and enjoys taking risks. She admits mistakes and fosters trust and commitment to the vision of the hospital.

What is the most likely leadership characteristic possessed by the medical director in this regard?

- A. Self-confidence
- B. Cognitive ability
- C. Business knowledge
- D. Honesty and integrity

ANSWER: A

186. Reports from a hospital indicate that there is higher neonatal mortality. The manager of the hospital arranged successive meetings to make decisions in order to solve this problem.

What is the first step that should be carried out in the decision-making process?

- A. Define the problem
- B. Analyze the alternatives
- C. Select the best alternative
- D. Develop potential alternatives

ANSWER: A

187. At the beginning of a new year, the drug management committee of a hospital reviewed the prevalent health problems in the catchment area and identified the drugs of choice and dosage forms to be available in the coming year.

What is the most likely phase of the drug management cycle explained in this case?

- A. Selection
- B. Procurement
- C. Distribution
- D. Rational use

ANSWER: A

188. A Regional Health Bureau planned to establish a health facility that is expected to provide primary-level health care to a population of 60,000 to 100,000.
What type of health facility is planned according to the current healthcare tier system of Ethiopia?
A. General hospital
B. Primary hospital
C. Rural health center
D. Urban health center
ANSWER: B
189. A surgeon and an anesthetist were performing a procedure for a patient. The surgeon just ignored the anesthetist's recommendations for the procedure and did the work his way. Consequently, the needs of the surgeon are met but not the anesthetist's.
What is the most likely bargaining situation applied in this case?
A. Win-win
B. Win-lose
C. Compromise
D. Avoidance
ANSWER: B
190. A project manager wanted to assess whether the project was progressing as planned or not. The manager wanted to collect information, identify gaps, and make immediate decisions about how activities could be improved, along with checking if things were going all right.
What is the most likely form of management control used by the manager of the project?
A. Monitoring
B. Supervision
C. Evaluation
D. Attendance
ANSWER: A
191. A health project run by a non-governmental organization provides outreach screening and food services to under-five children in selected districts. After five years of implementation, the project managers conducted a survey to document under-five mortality and found that it was reduced by 75 percent. The survey also showed that 20 percent of the reduction was attributed to the project.
What type of evaluation was conducted by the project managers?
A. Process evaluation
B. Outcome evaluation
C. Formative evaluation
D. Summative evaluation
ANSWER: D
192. A hospital was found to be delivering a low quality of health services. The most frequently stated problem is a shortage of drugs and medical supplies. Senior officials determined that the main cause of this problem was a lack of ongoing monitoring by the concerned managers.
What is the most likely component of monitoring not properly handled by the managers?
A. Input
B. Process
C. Output
D. Outcome
ANSWER: A

193. A 30-year-old married woman came to a psychiatric outpatient clinic for evaluation of persistent worry that had begun eight months earlier and was becoming increasingly disabling. She stated that she has difficulty controlling her worries, which disturbed her daily routine activities. Moreover, her worry was associated with muscle tension, irritability, difficulty of sleeping, and restlessness. She has no history of medical problems or substance use.
What is the most effective management for this woman?
- A. Lithium
 - B. Clozapine
 - C. Fluoxetine
 - D. Haloperidol
- ANSWER: C
194. After two days of her delivery by cesarian section, a 25-year-old woman became tearful, irritable, and hopeless for unknown reasons. She felt inadequate as a mother, and her husband has noted that she has no interest in caring for her baby. She had a similar episode during her first delivery.
What is the most likely diagnosis of this woman?
- A. Bipolar I disorder
 - B. Postpartum blues
 - C. Postpartum psychosis
 - D. Postpartum depression
- ANSWER: D
195. A 30-year-old male farmer was brought to a psychiatric outpatient clinic by his wife after becoming irritable for the past week for an unknown reason. During the interview, he reported that he was feeling excessively happy and felt rested after only three hours of sleep. On mental status examination, he appeared to be talkative, had flight of ideas, and was not attentive. His wife reported that he had not experienced similar episodes previously. He denied suicidal ideation and use of psychoactive substances.
What is the most appropriate therapy for this patient?
- A. Lithium
 - B. Diazepam
 - C. Fluoxetine
 - D. Haloperidol
- ANSWER: A
196. One week after the death of his father due to unknown medical illnesses, a 24-year-old man experienced a sudden onset of irritability, talking alone, and becoming aggressive towards other people. He heard voices calling his name and became suspicious. He has no history of psychoactive substance abuse and suicidal ideation.
What is the most likely diagnosis of this patient?
- A. Schizophrenia
 - B. Schizoaffective disorder
 - C. Brief psychotic disorder
 - D. Schizophreniform disorder
- ANSWER: C

197. A 45-year-old man came to a hospital for psychiatric evaluation after witnessing his friend being stabbed with a knife by a stranger six months before. He reported that, since the attack, he had been feeling nervous and experiencing distressing dreams related to the attack. He experienced some difficulty focusing on his work and difficulty of falling asleep. He was more concerned about the frequency of tearful episodes, sometimes brought on by hearing the name of his friend, but, at other times, occurring “for no particular reason.” He now felt extremely guilty for not helping his friend. He denied suicidal ideation or any other psychotic symptoms.

What is the most effective management for this man?

- A. Lithium
- B. Fluoxetine
- C. Haloperidol
- D. Methylphenidate

ANSWER: B

198. A 28-year-old male driver, who had sustained a car accident and was admitted to a surgical ward, was transferred to a psychiatric ward with complaints of hand tremor, nausea, vomiting, and difficulty of sleeping after the accident. He had not taken beer for the last two days which he used to use on a daily basis for the past five years. Twelve hours after he stopped drinking the beer, he heard a strange voice that communicated with him. He appeared to be agitated and tachycardic, but he was oriented and had no memory impairment. He has no medical problems.

What is the most appropriate therapy for this patient?

- A. Fluoxetine
- B. Diazepam
- C. Haloperidol
- D. Sodium valproate

ANSWER: B

199. A three-year-old boy was brought by his parents for a psychiatric evaluation for the concern of lacking facial expression and non-verbal communication as expected of his age. Upon comparison with other children, his parents noted that he seemed less interested in social interaction and social games with toddlers and adults. He insists on eating the same food every day.

What is the most likely diagnosis of this boy?

- A. Language disorder
- B. Communication disorders
- C. Autism spectrum disorder
- D. Intellectual developmental disorder

ANSWER: C

200. A 70-year-old male known hypertensive patient was brought to psychiatric services on referral by a family physician. His wife claimed that he had become so forgetful that she was afraid to leave him alone, even at home. He also gave up hobbies he once enjoyed and became increasingly quiet. One day while walking in an area he knows well, he could not find his way home. He failed to recognize people, even those he has known for many years. His wife had to start bathing and dressing him because he forgot how to do so himself. On mental status examination, he was disoriented to time and place.

What is the most likely diagnosis of this patient?

- A. Delirium
- B. Amnestic disorder
- C. Mild neurocognitive disorder
- D. Major neurocognitive disorder

ANSWER: D